

Draft Admission Action Plan

Resident Name: Resident A

AFH Name: AFH Operator

Date Prepared: May 15, 2026

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Status: Draft worksheet — requires operator, clinician, and family review

This document is decision support only. It is not a legal agreement, clinical order, billing determination, or substitute for the official negotiated care plan / service agreement.

1. Admission Recommendation

Recommendation: HOLD FOR REVIEW

Rationale: The risk register identifies a high-severity gap (gap_00) regarding 24/7 awake, line-of-sight supervision: the AFH disclosure frames this level of staffing as conditional — available "if needed" with an additional fee for private-pay or subject to ETR approval for Medicaid — while the discharge order requires it unconditionally for this resident's safety given moderate Alzheimer's dementia and impulsive unassisted standing attempts. This single high-severity gap triggers a hold_for_review under the deterministic logic, regardless of other factors. Additionally, multiple medium-severity gaps (gap_01 through gap_04) exist around PT coordination, environmental fall accommodations, sundowning behavioral protocols, and structured cognitive redirection, and an unresolved source disagreement on eating assist level (gap_05) affects safe meal-time staffing planning. Admission should not proceed until the awake staffing commitment is secured in writing.

2. Conditions Before Admission

- Obtain written confirmation from the AFH operator that continuous awake, line-of-sight supervision is provided as a standard baseline service for this resident, with no fee contingency or ETR approval delay before it begins.
- Confirm in writing that the AFH will coordinate and facilitate Home Health physical therapy visits 2x weekly per the discharge order, and amend the service agreement to reflect this commitment.
- Request the AFH operator document in writing (or via care plan addendum) that a low bed, bed alarm, and perimeter monitoring alarm are physically installed and will be maintained for this resident; verify during a pre-admission site visit.
- Ask the AFH operator to describe specific sundowning and agitation de-escalation protocols in writing, and document the agreed behavioral care plan in the resident service agreement before admission.

- Obtain a clarifying order or functional assessment from the discharging physician or a home health nurse specifying the required eating assist level, and update the care plan accordingly prior to admission.

3. Services / Supports Identified in the Care Plan

Dementia

- Provide frequent verbal redirection throughout the day using calm, simple, single-step language consistent with moderate Alzheimer's dementia and oriented-to-person-only status.
- Implement a structured late-afternoon and early-evening routine to anticipate and reduce sundowning behaviors, including restlessness, pacing, and visual hallucinations. Reduce environmental stimulation (noise, lighting changes) starting in the afternoon.
- Limit the number of caregivers entering the resident's room simultaneously. Default to one-at-a-time, familiar-caregiver interactions using a soft voice and slow, gentle approach during personal care.
- Leverage the AFH's dementia specialty care designation to guide all staff interactions, structured activities, and behavioral de-escalation approaches specific to Alzheimer's disease.
- Monitor for and promptly report any new or worsening acute confusion, fever, or behavioral escalation beyond verbal resistance, as these may signal UTI recurrence or other medical change.

Fall Risk

- Maintain 24/7 line-of-sight supervision at all times, with particular vigilance during any unassisted standing attempts or transitions.
- Ensure the front-wheeled walker is consistently accessible and resident is verbally cued to use it before any ambulation, including short distances to the bathroom.
- Maintain and verify all installed environmental accommodations (grab bars in bathroom, low bed and/or bed alarm, perimeter monitoring alarm) are functional and correctly configured at intake.
- Contact the ordering provider (or Home Health agency) to confirm current PT status and determine whether PT 2x weekly for gait stabilization should be reinstated or reassessed in the AFH setting, given PT was discharged over 3 months ago.
- Document and communicate any falls or near-falls to Jane (daughter, primary contact) during weekly phone check-ins, and escalate per discharge red-flag criteria for any actual fall.

ADL Support

- Provide limited/partial assistance for dressing, bathing, and toileting, using a one-caregiver, calm, unhurried approach to minimize verbal resistance during personal care.
- Offer finger foods as a default eating approach and provide verbal cues to support self-feeding. Treat eating assist as supervised/minimal pending clarification of the discharge eating assist level.

- Provide limited assistance and verbal cues for locomotion to the bathroom, with a light steadying hand as needed, ensuring walker is always in use.
- Supervision for transfers (bed-to-chair), with staff positioned to provide minimal assist if needed.

Medication Management

- Administer Donepezil 10mg orally at bedtime daily. Confirm this was the pre-hospitalization baseline dose before first administration.
- Administer Memantine 10mg orally twice daily. Confirm AM and PM schedule aligns with resident's daily routine and meal timing.
- Administer Acetaminophen 650mg orally every 8 hours as needed for mild pain. Document each PRN administration and observe for behavioral cues of pain given the resident's limited ability to self-report reliably due to moderate dementia.
- Confirm with prescriber or pharmacist that no fall-risk-increasing drugs (FRIDs) are present in the current medication list, and flag any future medication additions for FRID review.
- Schedule primary care follow-up with Dr. Smith within 7 days of AFH admission per discharge instructions. Communicate medication list and behavioral status to the follow-up provider.

4. Disclosure / Capability Gaps to Resolve

[HIGH] 24/7 line-of-sight supervision for safety due to moderate Alzheimer's dementia and ongoing fall risk from impulsive attempts to stand unassisted

- Missing or weak support: The disclosure states CNAs/long-term care workers are available '24/7' and that 'awake staff [is available] if needed w/ additional fee for Private-pay clients or through ETR for Medicaid clients.' This conditionality ('if needed,' 'additional fee,' 'ETR approval') means continuous awake, line-of-sight supervision is not guaranteed as a baseline service. The disclosure does not explicitly commit to 24/7 awake staffing as a standard — it is framed as an add-on. Resident A requires this level unconditionally per the discharge order.
- Suggested next action: Obtain written confirmation from the AFH operator that continuous awake, line-of-sight supervision will be provided as a standard service for this resident without fee contingency or ETR delay. If this cannot be guaranteed at baseline staffing, re-evaluate admission suitability or escalate ETR request before admission date.

[MEDIUM] Ongoing physical therapy 2x weekly for gait stabilization, ordered by the discharging provider via Home Health

- Missing or weak support: The disclosure references Home Health only in the context of skilled nursing services that cannot be delegated (e.g., IM injections, IV antibiotics). There is no disclosure language confirming the AFH will coordinate, schedule, or facilitate Home Health PT visits for gait stabilization. The disclosure is silent on PT coordination as a service.
- Suggested next action: Confirm in writing with the AFH operator that they will actively coordinate and facilitate Home Health physical therapy visits 2x weekly per the discharge order. Amend the disclosure or

care agreement to include PT coordination as a committed service prior to admission.

[MEDIUM] Structured environmental fall-risk accommodations (multiple modifications: grab bars, low bed/bed alarm, perimeter monitoring alarm) to address fall risk driven by impulsive unassisted standing attempts

- Missing or weak support: The disclosure does not specifically enumerate fall-prevention environmental accommodations such as low beds, bed alarms, or perimeter/door alarms. The only adjacent language mentions bedrails ('We can provide bedrails as needed to facilitate independence with proper assessment from professionals'), which is a distinct intervention. The operator's intake responses (OP17) indicate these modifications exist, but the formal disclosure is silent on them, creating a gap between operator representation and disclosed capability.
- Suggested next action: Request that the AFH operator confirm in writing (and update the disclosure or care plan addendum) that low bed, bed alarm, and perimeter monitoring alarm are in place and will be maintained for this resident. Cross-check the physical environment during a pre-admission site visit.

[MEDIUM] Behavioral management for sundowning (increased restlessness, pacing, visual hallucinations in late afternoon) and agitation/resistance to care in moderate Alzheimer's dementia

- Missing or weak support: The AFH holds a Dementia specialty care designation and the disclosure states activities are 'based on residents' choice, interests and cognition level,' which partially supports dementia-responsive care. However, the disclosure contains no specific language about protocols or approaches for managing sundowning episodes, late-afternoon behavioral escalation, hallucinations, or resistance to care. The designation alone does not confirm that a structured behavioral intervention protocol (e.g., scheduled redirection, low-stimulation evening routines, de-escalation techniques) is in place.
- Suggested next action: Prior to admission, ask the AFH operator to describe their specific sundowning and agitation management protocols. Request evidence of staff training in behavioral de-escalation for dementia residents. Document the agreed behavioral care plan in the resident service agreement.

[MEDIUM] Cognitive cueing and redirection support throughout the day for moderate Alzheimer's with orientation limited to person only, requiring frequent redirection per discharge summary

- Missing or weak support: The disclosure confirms dementia specialty designation and broadly states residents are allowed 'to perform tasks as much as they can with their safety in mind,' but there is no disclosure language committing to a specific frequency or structured approach for cognitive redirection throughout the day. This is particularly relevant given the resident is oriented only to person (OP4) and requires frequent redirection (S3), which demands proactive, scheduled staff engagement rather than reactive assistance.
- Suggested next action: Clarify with the AFH operator how frequent cognitive redirection will be integrated into the daily care schedule. Confirm that staffing ratios at this facility are sufficient to provide proactive redirection to this resident given other residents' needs. Document redirection frequency expectations in the individualized care plan.

5. Family Communication Notes

- We want to make sure your father has someone watching over him at all times, day and night — the hospital was clear that he needs continuous supervision because he can forget his physical limits and try to stand on his own. We need to confirm the home can guarantee this before he moves in.
- The discharge paperwork ordered physical therapy twice a week to help keep his walking safe. We're checking with the home to make sure they'll coordinate those visits, since the record shows PT hasn't been active for several months.
- We'll be verifying that the room is set up with a low bed, bed alarm, and door alarm before he arrives — those are important safeguards given his fall risk and his tendency to wander, especially in the evenings.
- You mentioned he tends to get restless and sometimes sees things that aren't there in the late afternoon. We're asking the home to walk us through exactly how their staff handles those moments, so we know there's a clear plan in place.
- There's a small open question about how much help he needs at mealtimes — the hospital notes focused on dressing and bathing, and you've told us he can feed himself finger foods with some prompting. We'd like a quick clinical check on that before finalizing the care plan, so the staff knows exactly what to expect at meals.

6. Open Questions Before Move-In

- Has the resident's baseline moderate dementia status fully returned post-delirium, or does the family or Home Health team observe residual cognitive decline beyond baseline?
- Should PT 2x weekly for gait stabilization be reinstated in the AFH setting, and if so, through Home Health or an outpatient referral?
- What is the resident's eating assist level per clinical assessment? Should an occupational therapy evaluation be requested to formally assess safe swallowing and self-feeding capacity?
- Are there any additional PRN or as-needed medications (e.g., for agitation or sundowning) that the prescriber recommends, given active sundowning with visual hallucinations?
- Does Dr. Smith's 7-day follow-up plan to reassess Acetaminophen use and determine whether ongoing PRN pain management is appropriate long-term?
- Does Jane (primary contact) have legal authority (e.g., durable power of attorney for health care) to make healthcare decisions on the resident's behalf, and has this documentation been collected for the AFH file?

7. Operator Completion Checklist

- ☐ Obtain written confirmation from the AFH operator that continuous awake, line-of-sight supervision is provided as a standard baseline service for this resident, with no fee contingency or ETR approval delay before it begins.
- ☐ Confirm in writing that the AFH will coordinate and facilitate Home Health physical therapy visits 2x weekly per the discharge order, and amend the service agreement to reflect this commitment.

- [] Request the AFH operator document in writing (or via care plan addendum) that a low bed, bed alarm, and perimeter monitoring alarm are physically installed and will be maintained for this resident; verify during a pre-admission site visit.
- [] Ask the AFH operator to describe specific sundowning and agitation de-escalation protocols in writing, and document the agreed behavioral care plan in the resident service agreement before admission.
- [] Obtain a clarifying order or functional assessment from the discharging physician or a home health nurse specifying the required eating assist level, and update the care plan accordingly prior to admission.
- [] Obtain written confirmation from the AFH operator that continuous awake, line-of-sight supervision will be provided as a standard service for this resident without fee contingency or ETR delay. If this cannot be guaranteed at baseline staffing, re-evaluate admission suitability or escalate ETR request before admission date.
- [] Confirm in writing with the AFH operator that they will actively coordinate and facilitate Home Health physical therapy visits 2x weekly per the discharge order. Amend the disclosure or care agreement to include PT coordination as a committed service prior to admission.
- [] Request that the AFH operator confirm in writing (and update the disclosure or care plan addendum) that low bed, bed alarm, and perimeter monitoring alarm are in place and will be maintained for this resident. Cross-check the physical environment during a pre-admission site visit.
- [] Prior to admission, ask the AFH operator to describe their specific sundowning and agitation management protocols. Request evidence of staff training in behavioral de-escalation for dementia residents. Document the agreed behavioral care plan in the resident service agreement.
- [] Clarify with the AFH operator how frequent cognitive redirection will be integrated into the daily care schedule. Confirm that staffing ratios at this facility are sufficient to provide proactive redirection to this resident given other residents' needs. Document redirection frequency expectations in the individualized care plan.
- [] Has the resident's baseline moderate dementia status fully returned post-delirium, or does the family or Home Health team observe residual cognitive decline beyond baseline?
- [] Should PT 2x weekly for gait stabilization be reinstated in the AFH setting, and if so, through Home Health or an outpatient referral?
- [] What is the resident's eating assist level per clinical assessment? Should an occupational therapy evaluation be requested to formally assess safe swallowing and self-feeding capacity?
- [] Are there any additional PRN or as-needed medications (e.g., for agitation or sundowning) that the prescriber recommends, given active sundowning with visual hallucinations?
- [] Does Dr. Smith's 7-day follow-up plan to reassess Acetaminophen use and determine whether ongoing PRN pain management is appropriate long-term?
- [] Does Jane (primary contact) have legal authority (e.g., durable power of attorney for health care) to make healthcare decisions on the resident's behalf, and has this documentation been collected for the AFH file?

8. Final Review Sign-Off

AFH operator review: _____ Date: _____

RN / clinician review, if applicable: _____ Date: _____

Resident or representative review: _____ Date: _____

Draft only. Final admission documents must be completed by the AFH operator using applicable DSHS-required forms and professional judgment.